Exam Case #4

CANDIDATE'S INSTRUCTIONS:

A mother approaches you for a recommendation for their child. Assess the situation and respond as you would in practice.

Time Frame: You have 7 minutes to complete this station.
References: CTMA (Diaper Dermatitis), 5 OTC products on table

SP instructions:

•SP has to look worried and anxious. He need some advise for his child.

•**Opening statement** “Hi, Could you please recommend something for diaper rash”?

•My name is Mary. Her name is Erica Martin.

•Upon asking, SP says ‘my child is crying quite often. Her diaper area is red. It looks very painful to see. Rash in her diaper area since last 3 days.

•**Only if the candidate** asks say. Can you describe your child symptoms of diaper rash? Say it is beefy red rash. If asked say I did observe a few satellite shaped spots. Only if asked say-No pus no boils, no macerations as yet. no fever or other symptoms, she had loose stools 2 days back now no loose stools.

•**Did you try anything?** Miconazole (Micatin 2%) and Zincofax twice daily.

•Only if asked? **How did you use it?** I applied Zinc Oxide first and then Miconazole.

•**Who recommended?** I picked up my self.

•**First time getting it-** well she had it when she was 1 year old. I don’t remember the name. It was suggested by pharmacist at that time and rash got better.

•If candidate asks about the medications, SP says her child is NOT on medication.

•Diaper change questions: **changes frequently, uses alcohol wipes cleans area,**

•Allergies: not known

•Medical conditions: none and healthy

•Active Healthy, No day care/school.

•5th min question? What are the risk factors for this?

•If candidate says “ I request you to go to the doctor”, SP says “I will. But can you at least give me something to help me right now?”

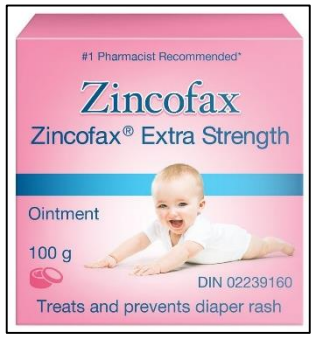
PATIENT RECORD

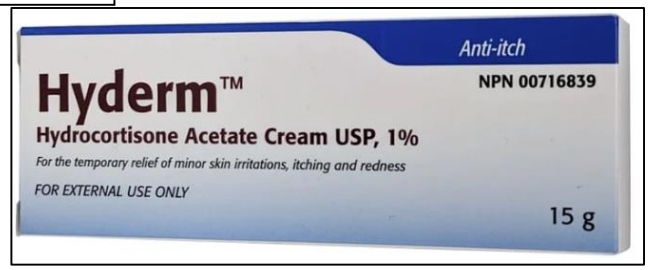
PATIENT: Erica Martin
ADDRESS: 7900 Pharmacy PREP Ave.
PHONE: 416-223-7737

AGE: 2 &1/2 years old
GENDER: Female
ALLERGIES: None known
Medical conditions:

PHYSICIAN: Dr. S. Mason
ADDRESS: #358-4750 Yonge st. Toronto
PHONE: 647-221-0457

Rx#	Medications-Directions	Qty	Repeats	
			Authorized	Remaining





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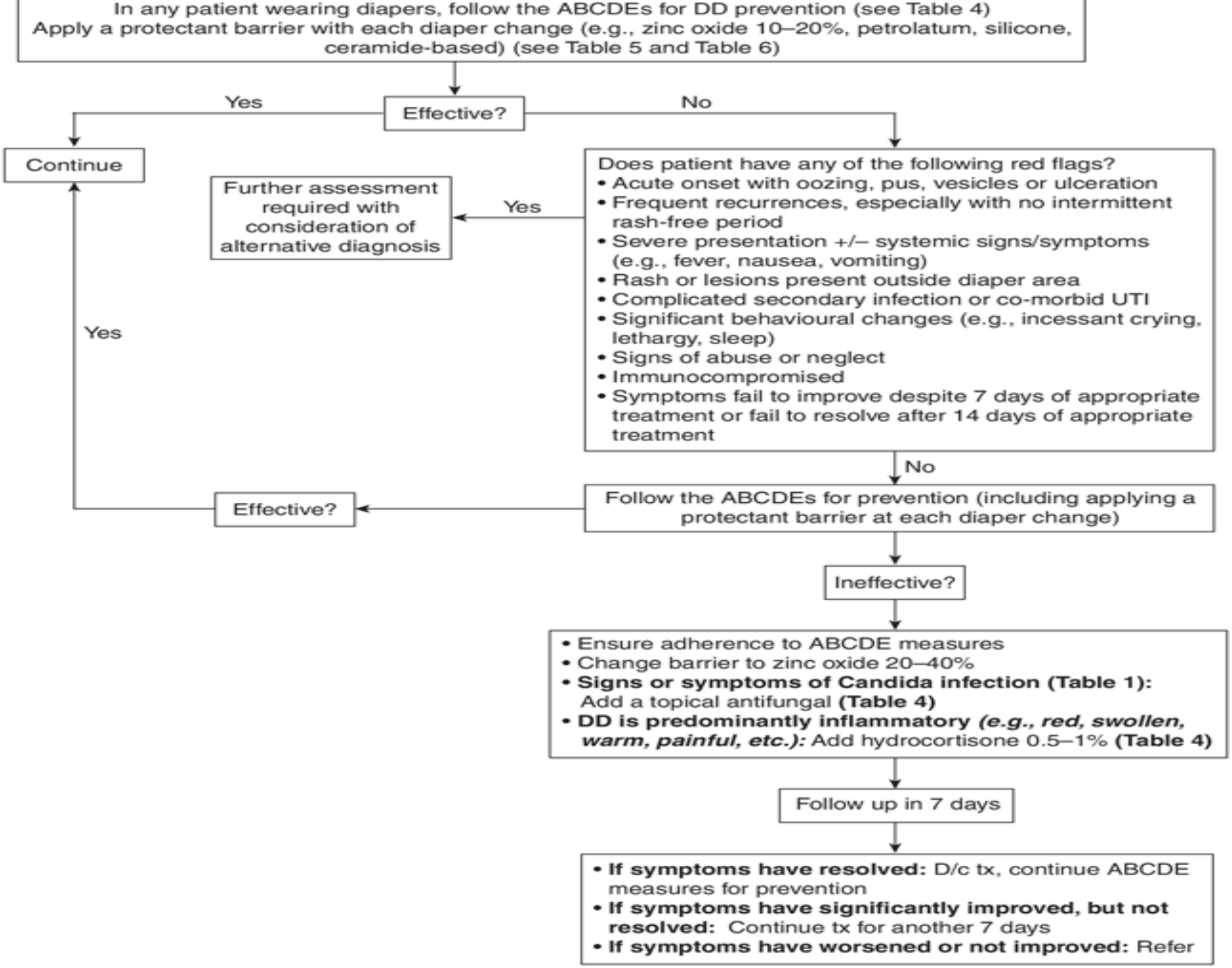
In any patient wearing diapers, follow the ABCDEs for DD prevention (see Table 4)
Apply a protectant barrier with each diaper change (e.g., zinc oxide 10–20%, petrolatum, silicone, ceramide-based) (see Table 5 and Table 6)
Effective?
Yes
No
Continue
Does patient have any of the following red flags?
• Acute onset with oozing, pus, vesicles or ulceration
• Frequent recurrences, especially with no intermittent rash-free period
• Severe presentation +/– systemic signs/symptoms (e.g., fever, nausea, vomiting)
• Rash or lesions present outside diaper area
• Complicated secondary infection or co-morbid UTI
• Significant behavioural changes (e.g., incessant crying, lethargy, sleep)
• Signs of abuse or neglect
• Immunocompromised
• Symptoms fail to improve despite 7 days of appropriate treatment or fail to resolve after 14 days of appropriate treatment
Further assessment required with consideration of alternative diagnosis
Yes
No
Yes
Effective?
Follow the ABCDEs for prevention (including applying a protectant barrier at each diaper change)
Ineffective?
• Ensure adherence to ABCDE measures
• Change barrier to zinc oxide 20–40%
• Signs or symptoms of Candida infection (Table 1): Add a topical antifungal (Table 4)
• DD is predominantly inflammatory (e.g., red, swollen, warm, painful, etc.): Add hydrocortisone 0.5–1% (Table 4)
Follow up in 7 days
• If symptoms have resolved: D/c tx, continue ABCDE measures for prevention
• If symptoms have significantly improved, but not resolved: Continue tx for another 7 days
• If symptoms have worsened or not improved: Refer

TIPS

SOLVED:

- Introduction, privacy, Red flags assessment, SCHLOR, MAMOVS, ADD
- Educate the patient: The application order of Miconazole is not right and that's why not effective. Ask pharmacist before using to avoid this situation.
- Recommend: Continue using miconazole, applied first and then barrier. After symptoms clear up, stop miconazole, continue using barrier.
- Counsel on Miconazole cream: Dosage, onset, side effects, Monitoring
- 5th min question: risk factors causing diaper rash
- Self-care
- Follow-up

MS:

- Introduction, privacy, Red flags assessment, SCHLOR, MAMOVS, ADD
- Did not realize about application order. Recommend to see dr.

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Pharmacy PREP OSCE Check List Case # 4

Case Title. Diaper rash

Communication skills (checklist)	**Outstanding**	**Good**	Acceptable	**NOT Acceptable**
Opening Statement (respond to patient clue, good listener, did not interrupt patient, engaging). Non-verbal Skills (eye contact, gesture, posture), Identified patient?	**4**	**3**	**2**	1
Information gathering process. (Asked M-A-M-A) allergies, medicine history, medical conditions. Builds trust and confidence with patient, Specific lifestyle such as alcohol and smoking habits prn; and/or other considerations relevant to the purpose of the case.	**4**	**3**	**2**	1
Discuss Follow up/Monitoring relevant to case	**4**	**3**	**2**	1
Time to check references, and use of proper references	**4**	**3**	**2**	1
Overall time management	**4**			
Overall communications	**Outstanding (20)**	**Good (15-20)**	Acceptable (12-14)	**(<14) NOT Acceptable**
Problem Solving (checklist)	Fully solved (FS)	Marginally solved (MS)	Marginally unsolved (MU)	Unsolved (U)
Educate the patient: The application order of Miconazole is not right and that's why not effective. Ask pharmacist before using to avoid this situation. Recommend: Continue using miconazole, applied first and then barrier. After symptoms clear up, stop miconazole, continue using barrier. Counsel on Miconazole cream: Dosage, onset, side effects, Monitoring 5th min question: risk factors causing diaper rash	All key points covered	3 key points were covered from 4 point	2 key points were covered from 4 point	Improper Recommendation

Misinformation?	**O**	**Put patients in to risk?**		**O**

Overall Performance	**Superior(O+FS)**	**Good(G/A +FS)**	**Fair(A +MS/MU)**	**Need to improve(U+MU/U)**